

Question:

A 62-year-old man presents to the emergency department with hematuria and hemoptysis that started in the morning. He notes that he has had frequent lung infections throughout his adult life, the most recent being 2 weeks ago. He also mentions that he has had hematuria twice before but never as severe as he is having currently. His medical history is otherwise non-significant, and his only medication is acetaminophen as needed. His blood pressure is 136/92 mm Hg, heart rate is 86/min, respiratory rate is 16/min, and temperature is 37.0°C (98.6°F). Chest radiography shows a resolving right middle lobe airspace opacity. His initial laboratory tests are notable for elevated erythrocyte sedimentation rate and C-reactive protein level. While in the examination room, the patient develops a spontaneous nosebleed. What is the most likely diagnosis? Answer Choices: A. Post-streptococcal glomerulonephritis B. IgA nephropathy C. Goodpasture syndrome D. Granulomatosis with polyangiitis

Reasoning + Answer:

<think> To approach this reasoning problem, we need to analyze the context of the question, which is centered around diagnosing a condition in a patient presenting with hematuria, hemoptysis, and a history of recurrent lung infections, alongside specific diagnostic indicators like a resolving airspace opacity and elevated inflammatory markers. The most relevant answer from the given choices is "Goodpasture syndrome," which is a type of rapidly progressive glomerulonephritis associated with pulmonary hemorrhage. This condition directly correlates with the symptoms described in the scenario.</think> <answer> Goodpasture syndrome </answer>

